Please include how you answered

MONDAY

General Tips/Advice

- You can have a scribble pad
- She doesn't like directly ask you "oh so what do you think about cl complications and hypoxia" → you have to lead the discussion there if you want to cover those points
- Does lily ask for a room tour - no, she just lets you know its recorded - can we use the blurr thingie on teams - my room is super messy does anyone know :((((
- Wait for her to call you/send link for teams (she might be running late from previous)
    - Does she send link on email?
- She will pretend to be a patient and you will kinda have to ask the questions yourself and walk her through your thought process
- She doesn't like you talking about the cost of lenses as an advantage/disadvantage. In an ideal situation you should be prescribing lenses based on other factors (e.g. Dk/t, lifestyle etc.)
- I swear someone else was in the call as well like another supervisor
- Lily's favourite colour is pink <3 so cute

Morning session

1. 44yo presbyope office worker (+1.00/-0.75x180) and how i would correct her for presbyopia and a basic -5.00/-2.25x180 -5.00/-2.25x145 and how I would fit the patient
    a. Why would you choose monovision/MFCL
    b. Presbyopia fit guide for new wearers
        i. >did she ask this bc the response was Biofinity/Proclear MFs? Since these are the only ones i think that come in D-centre and N-centre.

ADD	Dominant eye	Non-Dominant eye
+1.00	D	D
+1.50	D	D
+2.00	D	N
+2.50	D	N

        ii. CPV recommends ->
        iii. Is this what she means by presbyopia fit guide for new wearers? Any knowers :D
        iv. How does a D centre MF lens work?? If the pupil constricts for near tasks?
    c. Which lens would you give

2. 44 y/o presbyope needs glasses for reading, doesn't like glasses, what CL will you prescribe?
    - What else is available other than MF and monovision?
    - How does EDOF lenses work and give 2 examples - **can someone answer this pls (this is from my notes from lily's presbyopic CLs lecture)**

        - Uses several orders of spherical aberrations of defocus - in exchange for depth of focus - minimising ghosting and haloes
        - SEED1 day Pure EDOF
        - The other one is a virtual pinhole?? Naturalvue MF 1-day
        - It has a smoothly progressive plus power that increases towards the periphery, there's no spherical portion of this lens - it accelerates from the centre and increases until the optic peripheral blur creates like a virtual pinhole effect which the brain perceives and we get a EDOF in the eye -> therefore wide range of vision.
    - How do you work out eye dominance and is D and N the same?
        - +1.50 lens test at near to see which eye is clearer - i think not 100% sure. Asked that but then she was like does the +1.50DS make it more comfortable for dominant eye or not for near. I think yes?
            - **Is it +1.50 @ near over distance rx or with their add?**
            - **Think its over distance rx - ya**
            - She only goes thru it briefly in the lecture for presbyopes on moodle - literally only talks abt it for 5 seconds T.T
    - What would you want to know about this patient? RX, do they have any ocular problems, how many days will they wear the lens
    - How would you choose between SiH and Hydrogel? Hypoxia (time of wear, thickness of lens/high power)

3. 44yo presbyope comes in not wanting specs, prev optom said he needs Rx for reading. R +0.50/-1.00x180 L +0.50/-0.75x180 (Do yall want my answers to this even tho i know its wrong? **LOL) yes please 🙂 -**
    - What would you like to know about this patient?
        - Office worker, no Hx of prev or ocular problems, Hx of wearing coloured CL when younger
        - Plans to use it for 12h every day
    - What type of CL would you choose for this patient?
    - Why did you choose this type of CL for this patient?
    - Why did you choose MFCL?
    - What Rx and add for the CL?
    - Any alternatives?
    - **When would you not use H2O2?**
        - **What's the answer to this :(**
        - **I said coz u need to wait full 6hrs for it to neutralise, so if they use before then it stings. So px with bad sleep schedule is unsuitable.**
        - High water content lenses

4. Same as above q. 44yo presbyope comes in not wanting specs, prev optom said he needs Rx for reading. R +0.50/-1.00x180 L +0.50/-0.75x180
    - What do you do?
    - Which CL would you choose?
    - Which MFCL would you choose? Chose Myday MF DD. then she asked is it toric? Then said Biofinity MF monthly. She asked if i knew another one? I didn't.
    - How does the MF CL design work?

- What sort of problems with vision canu you expect from Biofinity MF toric lens? Halos around light since MF design, vision may not be as sharp.
- What other CL options are there? Said rigid lens but she proly wanted the EDOF lenses.
- How do you check for ocular dominance for distance and near? Said +1.50 dist target and dominant eye is one that is less comfortable. For near said same thing, then she asked me does it make more comfort or not for dominant eye again.
- What sort of considerations of the px do you make for monovision? Px loses binocularity with monovision
- What solutions do you use to clean biofinity MF monthly? And why would you choose either one?
    - **(Why would you choose a hydrogen peroxide system over a MPS system and vice versa?) - why :(**
    - **WAIT I THOUGHT H202 WAS FOR RGP ONLY???? O wait same wat :((H202 can be used for soft lenses**
    - Hydrogen peroxide is PF free so better that way
    - MPS since hydrogen peroxide not suitable for px with bad sleep schedule
- If px had +3.00 instead of +0.50 but the same cyl. Which px would have more benefit? What is the answer to this :C hyperope = happier? But they're both hyperopes i guess the more hyperopic guy is happier cos he has to converge less? now.
  Does it have anything to do with thickness/centration of lens with it being more plus?
  Idk i just said +3.00 person woulda had more benefit and she didn't probe further.

If power was like -5.00/2.25x180 OU. what power lens would you give? How did you calc?

- Aren't you meant to go more plus when vertex correcting? UH OOPS

<u>Afternoon session</u>

1. I am a patient walking into your clinic who has never worn CLs before. I would like to know more about them. What would you ask me and what would you tell me?
    - A whole bunch of initial consult questions e.g.
        - What's your age? 23 year old female?
        - Do you currently wear glasses?
        - What's sport/hobbies and occupation? Swimming, squash and I'm an accountant.
        - Do you use the AC a lot in the office?
        - Why do you want to wear CLs? I don't want to wear glasses anymore
        - How often would you be wearing them? Everyday
        - Explained the different lens modalities and accidentally mentioned that monthly lenses would be cheaper than dailies if you wore them everyday. Was probed on cost of lenses and when would DD be cheaper than monthly lenses. "What if the patient wore the lenses for 5 days a week, which one would be cheaper?" "What if they wore for 4 days a week only when playing sport?"
    - What would you do next to determine what lens to wear?
        - Check Rx
        - Check OcHx
    - Rx: OD +6.00/-1.00x140 OS +6.50/-1.00x60
        - What lens would you choose?
        - Why?
        - If you saw 10 degree nasal rotation OU with Bioinfinity toric what would you do?
        - What cleaning solution would you use? H2O2
        - When wouldn't you use it?

2. 23 year old female never wore contacts before and interested in CL.
    - What would you ask me?
        - Same initial consult questions as above
        - OD +6.00/-1.00x140 OS +6.50/-1.00x60
    - What lens would you give?
    - What lens would you give if she wanted to swim in them?
    - What would you advise for with bioinfinity lenses?
        - Cleaning of lens everyday, selected Optifree MPS solution
    - Why that solution? When would/wouldn't you choose to give hydrogen peroxide?
    - What things do you check for at a lens assessment
        - Aftercare: hypoxia
    - Why would you be worried about hypoxia for this patient?
        - Lens thickness
        - Wearing for long duration everyday
        - She kept asking what else

3. 44-year-old female, never worn glasses before, been told she needs SVN but wants CLs instead of glasses
    - Her Rx: R +1.00/-1.00x140, L +1.00/-1.00x60, not sure which add but she is 44 years old
    - What options do you have? Monovision, Multifocal CLs
    - What do you want to do next? Dominance test. How?
        - +1.50D over each eye while wearing distance Rx, both eyes open, looking at a distance target for D dom (and a near target for N dom)
        - Blurrier when RE has lens over it at distance → RE D dom
        - Blurrier when RE has lens over it at near → LE N dom

I answered it wrong - saying RE dom both D & N → monovision prolly not ideal option
→ Multifocal CLs
    - What do you do now? Multifocal CLs. Rx?
        - Biofinity toric MF R +1.00/-0.`5x140, L +1.00/-0.75x60
        - D centre design (according to CV Biofinity fitting guide), should have said +1.00D add too which is the lowest add
    - Why did you choose this lens?
        - I said smth like -1.00 cyl probably won't get away with just spherical equivalent, so I want a toric MF lens
    - What other toric MFs are there and why did you choose Biofinity toric MF instead?
        - Proclear toric MF - but this is hydrogel which is associated with higher risk of hypoxia
        - Biofinity is SiHy → higher Dk, less risk of hypoxia, also doesn't dehydrate as much as hydrogel → the better choice out of the 2
    - What modality is this lens and what do you need to do next?
        - Monthly
        - Prescribed hydrogen peroxide cleaning solution
    - Why? In which scenario is it not suitable to use hydrogen peroxide?
        - Why? - better cleaning action compared to MPS (unsure about this tbh)
        - Scenarios - as above, unsuitable if you can't soak it minimum 6 hours

4. 42 year old - same case mostly as above, but she asked me about adverse events related to MPS vs hydrogen peroxide.
    - Why is hydrogen peroxide better than MPS?
    - Adverse events of CL wear when using MPS?
        - I wasn't really sure what she was asking for… and i think it might have been related to maybe allergy to MPS?
        - I went in a circle and got to solution toxicity but not sure

5. 44 yo, never worn glasses before. Wants to wear CLs, has recently been told they need reading specs ^same as above q's

- Same questions as above, but also asked about:
    - What lens would you choose - I said Biofinity or Myday depending on use, she asked me to explain why I would choose those
    - When would and wouldn't you use monovision
    - What would you recommended for cleaning reusable lenses
        - I said MPS cos of convenience
    - She asked when I would use hydrogen peroxide instead and when wouldn't I use it?

No Tuesday Session

WEDNESDAY

Morning session

1. I am an OK wearer for 20 years but i do shift work now so i don't get the full sleep period.
    - What would you want to ask me?
    - How would you start me on soft contact lenses?
    - How do you know when the wash out period is done?
    - What soft lens would you recommend for me?
    - What are we worried about with extended wear?
    - Can you name some extended wear options?

2. I am a 30 year old OK wearer for 20 years, never been to see you. I do shift work at the hospital and get irregular sleep. I am getting fluctuating vision, can you help?
    - Wants to wear CLs everyday.
    - I asked if the vision gets better after say 8hrs of sleep? Yes it does.
    - What are you going to do for this patient? I said do an OK washout and refit with a monthly CLs
    - Why monthly CLs?
    - How do you know when you can refit the patient? When the topographic maps are the same after 2 visits (need 2 consecutive stable maps) then you can refit.

- **I also asked when the px doesn't wear the OK lenses for 1 night how is their vision** - Lily: Very bad. Then she asked me how do you know what 'very bad' is. Lol dont dig urself a hole.
    - What contact lenses would you give this patient in the meantime? When would you see them again? I said dont give px anything today but see them again tmmr. *px comes back tomorrow, their rx is now -2.00D, what do you do?* I gave -2.00D for today, unsure what to do next but i said give a couple pairs of -3.00 and -4.00 and see them in a week.
    - Would you want to give spectacles to this patient? Yes bc if they have red/irritated eye = wear specs.
    - What monthly CLs would you give?
    - Why are you wanting a high DK/t for this patient?
    - Would you let this px wear the lens overnight (EW)? I said no bc corneal hypoxia
    - Then she asked about hypoxia and what it causes to the cornea - corneal edema (she asked how do you see this clinically?), microcysts, folds (present when >8-10% edema), striae (present when >5% edema)

Ty i hate formatting <3

3. Same as above
    - I said come back in 3 days for some lenses, then 1 week for more lenses then another week to check topography LOL SHE ASKED WHY 3 DAYS. JUST SAID FROM WHAT IVE SEEN AHAH
    - I chose DD lens- acuvue oasys 1-day, she asked why acuvue oasys?
    - What complications with these lenses? What did u say for this!
        - High modulus CLPC? + cost
    - What complications does it avoid in DD?
    - Would DD reduce infection risk? I said yes cos lenses and cases
    - What if I want to sleep in the lenses? EW
    - What EW lenses? What else x3 i named like 3 and she kept asking, i wanna sink

- She dug me a hole by asking me about acuvue oasys fortnightly LOVELY. Yeah ik nothing about it rip
- What do you want to tell me as a previous OK wearer, now going to wear soft lenses? Man had to go through both DD and EW. Talked about I+R, hygiene, wear modality etc

4. Similar case as above - 30yo recently changed jobs, “orthok not working anymore”
    - Awkward silence man i had to ask her why is it not working is it coz lack of sleep. Lily: YES
    - What would you do for this patient - washout + fit SCL (i talked about DD and why I chose it)
    - What would you give during washout - trials but idk what powers like 0.50steps each day??
    - When would you review - 2 wks? Possibly 1 week to determine what Rx trials to give for the next wk and monitor washout
    - After washout what would you do that that visit - topo, refraction, make sure back at baseline
    - Say you fit them with a monthly lens, what solutions would you use - I said Optifree vs Aopsept, but in this case Optifree → she asked why: coz no need to wait 6hrs to neutralise (optifree is also 6 hours disinfection i believe - yes)
    - Why H2O2 better than MPS - MPS contains preservatives → causes SICS, SLK, allergies
    - Staining pattern with MPS - diffuse, epithelial
    - What are signs of hypoxia - listed a few
    - THE HOLE BEGINS - how do we look at endothelial cells → I said imaging lmao then she asked what about on SL → specular reflection
    - How do we set up specular reflection on slit lamp - light and observation system 45 deg?? (ACTUALLY 60 DEG)
    - What mag do we need to look at cells - i said 25x oops
    - How thick is the endothelial layer - 5 microns (is this a slit lamp oral smh)
    - Can you list a EW lens in the market - Biofinity for 30 days 29 nights
    - I forgot the rest =)

5. Same case as above (she knows we are telling each other answers, the qns are getting more and more specific)
    - Name two H2O2 methods and their adv vs disadv
    - What are the options for EW? How long are they approved for each?

Same as above.

- why I chose biofinity for EW, and when i would dispose of it- and i said its approved for 7 days of EW (which is wrong), and that i would dispose after 2 weeks
- why 2 weeks - I wasn’t sure.
- what other options are there cos that would be expensive. I said either Alcon DT1 DD, or Air Optix Night and Day which is approved for EW 30 nights. She kept asking what would be better price wise but i wasn’t sure. T
- Why did you choose biofinity - i said good Dk/t. and then she asked what the specific value (i was wrong)

6. Same case as above: 30YO Ortho-K wearer for 20 yrs (wore them last night), recently changed job, OrthoK now not suitable
    - Corrective options?: i said specs and CLs (DD & Monthly)
        - Made me talk about CLs (urgh)
    - Asked about ortho-K washout and Mx → I said give DD SCL while cornea is regressing to pre-treatment status and takes 3 weeks to 2 months
        - What CL would you give the px today? (i said -1.00 to take home)
        - Do you think the Rx is -1.00 on the next day? (i said it could be, but depends on the extent of the pre-treatment Rx, can be higher or lower, if it is lower, then just dont wear CLs; ie. there could be some residual ortho-K treatment next day)
    - Asked about correction options post-washout (for a -6.00 Rx)
        - I said can give DD SCLs or monthly SCLs - *why not fortnightly?*
        - Lily said to name a specific monthly SCL → i said CV Biofinity Sphere in -5.50D
        - Why this lens? → monthly lens more similar to ortho-K compared to DD. Biofinity has high Dk/T (160), low modulus (0.75) and good power range options. Said to also consider topography before choosing CLs as I havent been given much info lmao
        - What could be a problem with wearing Biofinity? [i said idk, bc lily was fr gonna go digging]
    - Lets go with Proclear as an option
        - What are some complications of proclear vs complications from biofinity → talked about SiHy vs Hy (modulus, Dk/T, subjective comfort)
        - What is a benefit of proclear > biofinity → said modulus is lower, which can reduce complications associated with mechanical lens-lid interactions e.g. SEAL, CLPC
        - What is a complication you may see in SiHy that is not in Hy that isn't related to the modulus → i said idk lmao, would it be lens fluting?
    - Ok, so now you have fitted this lens on the eye (Biofinity) its fully stablised etc., what are you looking for?
    - Ok so you mentioned CLPC before, what causes CLPC? What is it?
        - Allergic, mechanical
        - What else? I said trapped deposits
    - What deposits can you get on biofinity? And what about on proclear?
        - Protein on proclear, lipid on biofinity
    - How do you see these deposits/what do they look like? Where would you see them?
        - Idk lmao
    - How would you check for deposits on the lens?
        - I said remove lens and check on slit lamp, but i backtracked and said nah check it on the anterior surface of the eye

7. Same case as above: 30YO Ortho-K wearer recently changed jobs and is working at a hospital. Their vision is fluctuating with the orthok and they dont want to wear glasses.
    a. What CL would you advise?
        i. She mentioned patient gets varied number of hours of sleep
        ii. CV MyDay bc its DD and dont have to worry about spending time cleaning it esp if they dont know how long they get to sleep for with their shift work. Since they are an orthok wearer, their Rx would change as it returns to baseline
    b. Why would you want them to wear DD instead of monthly if they wear the CLs everyday?

    i. Kinda fumbled and just said they get more utility and cost advantage of using the DD, because their Rx will keep changing until it returns to baseline
c. Sometimes they like to nap in the CLs, e.g. for an hour during work. What CL would you give them?
    i. Advise them not to nap in DD and consider giving Biofinity since they have 29 nights EW
d. Why would you advise them not to sleep in the CLs?
    i. Has a higher chance of leading to a corneal infection because bacteria can proliferate in the moist closed eye environment (should have also mentioned no tear pump to flush out endotoxins and stuff). I also mentioned something to do with hypoxia but she didnt take the bait sigh
e. What is the risk of infection if the patient sleeps in the CLs (sounded like she was looking for a specific number)?
    i. I said idk but there is an increased risk and then we moved on lol
f. If the patient wants to sleep in the CLs, what can you do to ensure it is done most safely?
    i. Played some solid thinking in my head for 5-10 secs before i said we should review the patient more often idk but she seemed okay with that
g. What solution would you use for the biofinity?
    i. ReNu bc it has shorter period of soaking (4 hours) compared to other brands like OPTI-Free (6 hours)
h. What are risks of MPS?
    i. (didnt specifically say risk but something along those lines)
    ii. Can lead to solution induced corneal staining if the patient is allergic to the preservative Dymend/PAPB in the MPS. In this case, we could swap them to another MPS, such as OPTI-Free which uses a different preservative.
i. What does SICS look like on the cornea?
    i. Bilateral diffuse corneal staining, assuming they wear the CLs on both eyes
j. Gave me some Rx i cant remember and asked me what i would prescribe in terms of Rx and lens and why

Afternoon session



NO RX OR ANYTHING FROM THIS WOMAN

1. **CC: “I want to go camping, and I thought I would like try to CLs instead of glasses”**

I asked if they had any prior experience in wearing CLs and if they planned on wearing these CLs strictly for camping purposes or even on a day to day basis. She replied, “mmm if they’re good, I’ll continue to wear them.”

First suggestion: Daily Disposable Lenses as they do not require any maintenance such as solutions or lens cases which would be ideal in camping scenarios.

- What would tell the patient to do for camping?
  Daily disposables, so just insert them and take them out everyday.
  (LOL i now realize, I probably should've told him they MUST be discarded I was very general about this)

- Make sure you wash your hands before lens insertion and removal - if the water source is not as clean as in sydney (metro), still wash hands but make sure they are completely dry. LOL i said make sure they don't get on the lens as well - but they are DDs

- Asked me something about **lens handling** - I said DT1s are silicone hydrogel so have higher Dk/t and are naturally higher modulus material, and therefore should be easier to handle than hydrogel lenses due to increased stiffness.

What if there is **NO water accessible:**

- EW option would be the best e.g. Acuvue Oasys (1W EW)
- Thought monthly wouldn't be necessary unless they are going on a long-term trip

What would need to tell the patient in regards to EW?

[On the assumption that he will be not be taking them out I said]

- Alert the patient of the inherent risks involved with extended wear such as CLPC or CLARE - if he experiences red eye, that is mildly irritated or painful, he needs to take them out and use his spare glasses (tell him to bring spare glasses).
- Otherwise I couldn't think of anything specific about camping and EW
  EW lenses typically have good dk/t too (including Acuvue Oasys and she didn't even tell me if this was a spherical or toric rx)

What would you be looking for in a CL fit?

- Lens centration
- Lens tightness
- Lens movement - ensure there is enough movement for tear exchange
- I would also look at anterior health to make sure he is suitable for CL wear in general first too.

Lily: "Let's say your EW CL fit was good" when would you like to review him?

I could not think of reasons why I would review him the next day so I told her 1 WEEK !

- Would ask the patient to actually sleep in them (same amount of nights as camping ideally) so i can replicate the situation he would be experiencing and then review in the weeks time
- I would also have to teach him insertion and removal properly + CL disinfection and storage since he is a first time wearer.
- I am hoping in the one week review I should be able to tell whether EW contact lenses are suitable for him before he goes off camping
- Lily: "So you're saying, after you fit him in EW lenses you would review him in 1 week time and ask him to sleep in it a few times during that week?"
- ME: YES.

"Is there anything else about EW you would tell your patient"

No sir :(I could not think of anything else I needed to tell the patient

- Maybe she was looking for me to explain to the patient what EW and DD were? I am not too sure

2. Wants to go camping for 1 wk in 1 months time in forest, never worn CLs, dont want to wear specs cuz it might fog up, no inf/allergies/bleph/dry eye
    a. What can u give him? Said DD at first
    b. what if no water, whats the problem with using DD? I said can get dirt when taking lens in & out, EW Coopervision Biofinity
    c. Why would you give him coopervision biofinity?
        i. Bc its high dk/t, moderate modulus so easy I&R for first timers, sihy
    d. What other lens can you give him? I said acuvue oasys fortnightly
    e. She said why wont u give him that and whats the diff b/w those 2
        i. I said modality lol honestly didnt know what else
    f. What would you do in the consult - tell me the process
        i. VAs, refraction, topo, SL, etc
    g. Gave me Spec Rx: R: -6/-1x125 L: -6/-1.50x45
        i. Spent ages tryna vd that shit ngl
    h. What power would u give him to try^
        i. Said R-5.75/-0.75x120 L -5.75/-1.25x40 at FIRST bc biofinity toric doesnt come in 5deg steps
        ii. Then she asked why u choose that axis: (i just said cuz it was the closest thing lol and i would trial it and see it if rotated and check their vision and use LARS to compensate if it does)
        iii. What rotation would you expect to see??
            1. I said nasal
        iv. Then she said then what would you give him
            1. I corrected to R:-5.75/-0.75x120 L -5.75/-1.25x50
    i. Aftercare? 1 wk
    j. Would u make him wear biofinities as EW in that wk before aftercare?
        i. Idk the answer to this but i said yes…just to see if it works for him
    k. What would u be looking for at aftercare
        i. Inflam, infection
        ii. Dont say CLPC i accidently blurted it out HAHAHA then she woz like does EW necessarily mean ull c CLPC smh - wait actually it does..
    l. If after they wear the EW for camping, do u still want them to wear the lens?
        i. I said to chuck them out LOL/depends on if they still want to wear glasses/CLs after their camping trip…,,.
    m. Omg and then i think i remembered at the VERY end she asked if EW didnt work for him what would u give
        i. I said id go back to DD and make him take some clean sterile water and soap and paper towels or hand sani with him which is a very dumb answer that i pulled outta my ass HAHAHAH BETTER THAN NOTHING

3. Same as above :')
    a. I did the exact same DD originally (DT1) - she asked why
        i. Listed off a whole bunch of deets here about DT1
    b. She said what other lens would you think of or what other considerations. I said i would consider if they had acces to sanitation or a way to clean their lenses
        i. Then said acuvue oasys fortnightly because he was only going to wear for 1 week during the camping trip and EW for the entire camping trip
    c. Then she asked me, this guy wants to do **watersport activities** during the camping trip, can you still give EW

        i. I said preferably no CLs because of contamination with water and microbes, goggles if he wants to wear the CLs
        ii. I said that i would get him to wear EW before and during the water sports and then have a spare pair of CLs that he could pop in after waiting 15 minutes and take out the old lenses
    d. Then she asked me again, so can he wear EW lenses? Basically emphasised again that he has no access to sanitation, how can he safely insert and remove the lenses.
        i. And i was like oh crap - said can wear for the entire trip but wear goggles and warn sign and symptoms of MK
        ii. Throw out the lenses at the end of 1 week (even though it is a fortnightly)
    e. Then got the same spec Rx as number 2
        i. Did the same type of principle - nasal rotation of the lens on the eye (but forgot that left eye nasal is add instead of subtract) - didnt correct myself
        ii. Its hard to multitask (speaking and calculating at the same time)
        iii. Basically asked why i back vertex corrected -6 and -7.50 the same (and i was like uhhh rule of thumb with BVD correction) - then went on to say that i expect the higher the power the more back vertex correction there will be, so it might be closer to 0.75D correction
    f. **note make sure you got good internet - my call cut out halfway through and she asked me to repeat everything again !!!!**

4. Same as above
    a. Camping scenario
        i. Asked Lily about the duration of camping, access to water, environment, etc. - same details as above
        ii. Said EW (Acuvue Oasys fortnightly, 7D EW approved since 1wk trip) and not DD bc no access to water → elaborate further
        iii. *I think this section didn't take as long for me cos I went straight to saying EW - and then I just had to give a general discussion on why not DD
    b. Fitting EW
        i. How do you fit EW? Talked about SL fitting parameters on initial consult etc.
        ii. When would you review? 1 week aftercare if camping trip is for 1 week (to simulate) - check for signs of hypoxia, lipid deposition, etc. (kept it vague)
    c. Spec Rx +5.00/-2.25x130, +5.00/-2.25x35
        i. What Rx? Which lens? Why?
        ii. (Explained my thought process/calcs as I was doing the calcs, screwed up the first time round cos can't maths HAHA but she kept asking what the Rx was so you could tell it was wrong lmao)
        iii. +5.25/-2.50x130, +5.25/-2.50x35
        iv. → go with lower cyl, +5.25/-2.25x130, +5.25/-2.25x35
        v. Then got biofinity toric/toric XR cyl power & axis ranges mixed up so Lily was like 'what's the rx range for toric XR' and then I was like ah shit let me correct myself
        vi. Went with LE +5.25/-2.25x40 → asked about why x40 (nasal rotation in LE, LARS)
5. Same as above
    a. Literally same as above, no new questions except:
        i. Name 3 diff brands of EW
        ii. MPDS vs peroxide
            1. How would you choose which MPDS to give?

iii. “What happens to the lens after 1 week of wear?” → idk i said u needa chuck it out
iv. “So basically the life of the lens is halved?”
v. “Why acuvue oasys for a 1 week trip as opposed to other lens?” ??idk i jus said its compatible with the length of his trip???
vi. Kept asking me what lens i would use even though i said Acuvue oasys like twice already - idk if she jus forgot or if she wants smth else

6. Wants to go camping for up to 2 weeks
    a. What would you recommend?
        i. Can do daily disposable as that’s most healthy for your eyes
            1. Acuvue Oasys
                a. Why do you want a high Dk/t for camping?
                    i. Don’t want high Dk/t for camping, it’s just in general I would want to be recommending a breathable lens
        ii. What else would you recommend?
            1. I couldn’t fake the fact that I already knew the question, it was so awks 😣
            2. I didn’t specify lens names, I just said a lens that you can keep in your eye and you don’t have to take out
    b. She also asked about can i use hand sanitizer - was stuck on this for SOOO LONG but didn’t give the answer she was looking for 😕
        i. I said hand sanny doesn’t seem to be as effective as full handwashing, not recommended by the government guidelines for hand washing, also idk if the hand sanny stays on fingers then you touch your eye then gg burnnnn. She said that’s true but what else
        ii. I literally stared into space and ummmmmmmed for so long wasted time smh
        iii. She said is hand washing or hand sanny better
            1. I said hand washing because of friction and bubbles = surfactant?? Allowing more surface area for the product to clean your hands
    c. Gives me script
        i. R +6.00/-2.00 x 130 L +6.00/-2.00 x 35
        ii. What lens would you give
        iii. I went down the EW route, Biofinity toric monthly 29 nights EW approved
        iv. I VD’d it wrong (went more minus instead of plus). Lily saves me and goes, how did you get +5.50? Im like crap thanks lily
        v. Final rx: +6.50/-2.50 x 130 or x 40 (explained the whole nasal torque thing), but would need to change the cyl to -2.25, which yes is available in Biofinity toric monthly
        vi. She goes can you please tell me the power range for biofinity toric
            1. I say for sphere of +8.00 to -10.00 you can have -2.25 from 10 to 180 at 10 deg steps
                a. But then now i realised i didn’t explain -0.75, -1.25, etc cyl lol
                b. She was like …. So i think i was wrong/didn’t get what she wanted?
    d. What solutions would you recommend bringing?
        i. I’m like huh? Why would you bring solution, i thought we were going for EW and not taking the lens out
        ii. She goes, well, I want to listen to your recommendation

iii. I'm like ok, I'd rather you keep the lenses in and not touch with your grotty hands, BUT iF for some whatever reason the lens falls out or you wanna just clean it (but also gotta display my knowledge) here goes
iv. Gold standard is hydrogen peroxide eg. AOsept, just chuck the lens in and let it neutralise
v. BUT thinking about dirty forest I am worried about acanthemoeba so I am thinking Optifree Puremoist might be better because it has action against acanthemoaeba, BUT then that means you gotta be rubbing and rinsing with your grotty fingers which you apparently cannot sanitise so it's all a balance really
vi. Ie. my recommendation is to keep the lens in and don't touch with your yucky hands

e. Thanks that's all we got time for today LMAOO **I think she was after lubricants?**

7. As above: 1st time wearer, 3-4 days of camping
   R: +6.00/-2.00x130
   L: +6.00/-2.00x35
   a. What would you recommend
      Originally said Acuvue Oasys for astigmatism, but realised that it doesn't come in that Rx so I chose Biofinity torics as EW
   b. What is the script you would prescribe?
      R: +6.50/-1.75x130 (ah lmao i did the maths wrong xd)
      L: +6.50/-1.75x40 → lily asked why 40 and not 30 (idk something about nasal torque. In hindsight maybe I should have said to get them to trial it out first and check the script?)
   c. What would I do with the lens after the camping trip?
      I said throw that lens away just in case of risk of infections from camping environment. → lily asked me how many there are in a pack (3), idk if she was prompting me to say to re-use it with solutions?
   d. Moving on, what solutions would you recommend?
      AOSEPT → what other hydrogen peroxide system are there? (I described the neutralising tablet one but forgot it's Oxysept) (national shortage of AoSept)
   e. Why did you choose AOSEPT over MPS?
      AOSEPT is Preservative free → when would you choose MPS over AOSEPT (I answered in the event where you don't have enough time to clean for 6h at night and lily asked what else but i didn't know :( )
   f. She told me to name a few MPS solutions
      Optifree puremoist and biotrue → lily asked about what problems I would expect (SICS) → describe what you would see (diffuse punctate epithelial staining) → is that always the case (yes??? IDK :(((()

8. Same as above
   a. R: +7.00/-2.00x130 L: +7.00/-2.00x35
      i. Spectacle rx: did it based on rule of thumb (for a 11mm vertex distance, 4.00 → 0.25, 6.00 → 0.50, 8.00 → 0.75, 10.00 → 1.00)
      ii. Cls rx: R: +7.50/-2.25x130 L: +7.50/-2.25x35
   b. What lens would you prescribFRe?
      i. Biofinity toric XR because I want to correct for the 35 axis since it's a high cyl <u>isn't this only avail in +8.50 and higher sph? You're right rip me</u>
   c. What problems would you get with a lens with this prescription?

i. Needs very good prism stabilization techniques since pt relies on good cylindrical correction
ii. Mentioned Biofinity toric XR has peri-ballast (wide ballast band) so inferior portion of lens will be much thicker → lower dk/t → regional hypoxia → important to check Fl staining at each aftercare
iii. If inappropriate fit or staining and full frequent blinks etc aren't helping then fit another type of lens (if she probed i would say another SCL toric stabilization technique or RGP)

THURSDAY

Morning session

1. I'm a new patient who has never worn CLs before. What would you ask me?
    - Wants to wear for sports: basketball, soccer and tennis, also wear to the beach
    - Given script: -10.00/-2.00x135, and -10.00/2.00x45. What lens would you give me? Why?
    - Am I a candidate for RGPs? Why/Why not?
    - Advantages and disadvantages of RGP/SCL
    - Could I use fortnightly/monthly lenses?
    - What lens?
    - What other instructions for fortnightly/monthly lenses?
    - Ook that's all we have time for folks, enjoy the sunshine!

2. Same patient as above;
    - Also asked monthlys vs dailies
    - What solutions to use, H2o2 vs MPS
    - Adv and disadv of H2o2
    - Asked for all SCL available for that rx

3. Same patient as above
    - Asked if they planned to wear everyday, would you recommend monthly lenses?
        - The prescription correction → LARS (cause I did it wrong she asked me to explain why did I subtract and add but forgot to correct myself after :(
        - Asked me what bacteria are you concerned with when swimming?
            - Is it just acanthamoeba? (Yea, I said that but then didn't know what to say after… cause she was like what if they still want to wear the monthly lenses after swimming, and i said hmm h202 doesn't kill acanthomoeba…
    - What RGP would you fit? → what do you expect on the topography?wjich one did ya say please
        - I said corneal cyl, then she said other than that what else? I had no idea sorry :(
    - What solutions to use for rigid lenses
    - What other options could you do instead of myday toric?

4. 30 YO who wants to wear CL for sports because specs are annoying.
    - What would you ask?
        i. How many days they want to wear the lens → 3-4x/week
        ii. Prescription
        iii. Ocular allergies, infection or dry eye
    - What lenses would you recommend? → said DD which provides best safety and convenience
    - Same rx as above (pretended that I calculated it on the spot) → recommended MyDay Toric
    - Asked why I recommended DD
    - When would I not recommend DD
    - How many times would the patient wear the lens for me to not recommend DD
    - If I would like to go swimming, what would you like to advise me
    - I am interested in RGP, what would you say/ask me?
    - What complications do you expect to see in RGP

    - What solutions to use for RGP
5. Pretty much same as above lol
    - -10.00/-2.00x135
    - -10.00/-2.50x35
    - Only thing different is that LH asked about if you'd be concerned about this lens/pt
        i. I said maybe 30yo demographic is more risk tolerant, hence maybe pt could be more lax with hygiene etc.
        ii. Answer was about hypoxia because its a thick lens but eventually got there in the end
    - She asked me about when DD wear are more cost effective than monthlies
        i. Is it up to 4 days?? **<5 days DD more cost effective**

6. 30yo interested in wearing CLs. Recently started playing a lot more sports. Can you help me with this?
    - I said most definitely Lily :D lol
    - Started asking my questions re: CL wear
    - Gave CL Rx: R -10.00/-2.00x130 L -10.00/2.00x45. What options do I have?
        i. I gave soft CL options
    - Do you think this patient is suitable for RGPs?
        i. I said yes, explained why
    - What are some reasons you would pick RGPs in general for a patient?
    - What are some RGP materials?
    - How do you clean RGPs?
    - That was legit it. She then said "I think that's all we have time for today, I hope you enjoy the rest of your day."
        i. And I said "I hope you have a fine day too Lily!"
        ii. She laughs LMAO
        iii. The end

7. Same case as above.
    - Spec Rx: R -10.00/-0.50x135 L -10.00/-2.00x45
    - I also asked about corneal topographies→ she just said they were normal lol
    - Talked about why i would choose DD over other lens options briefly
    - Picked a SCL then Lily asked what would you talk about with this patient if he was interested in Rigids
    - Asked me about different RGP lens cleaning options

8. Same case as above, but wants to wear them all the time.
    - What questions would you want to ask?
    - Spec rx: R -10.00/-2.00x135 and L -10.00/-2.00x45
    - Asked about their suitability for RGPs
    - Pros and cons of soft vs rigid
    - Asked me what cleaning solutions to use for rigid lenses
        - Advantages and disadvantages of each
    - Asked for different rigid materials and the difference between them
        - I said Boston XO and Boston XO2 and difference is dk/t but she asked for other differences (I wasn't sure)
        - Asked for other rigid lens parameters you would consider when choosing a rigid material for RGPs

        - High dk/t is not recommended for ex PMMA wearers who are rough with their lenses, bc high dk/t in a RGP makes it very thin
        - it also has poorer wettability so maybe not for people with abnormal tear films? Or dry eye?

9. Same as before
    - Rx: OD -10.00/-2.00x180 OS -10/-0.75x35
    - CL H&S
    - Discussed between soft and rigid CLs (adv and disadv of each)
        - Rigids easier dislodged in sport due to smaller diameter
        - Potential issues with fitting CLs in general → hypoxia, discomfort from lens thickness of high Rx,
    - Name RGP materials and what's the difference?
    - Cleaning solutions for both soft and rigid
        - How would we choose between MPS for soft CLs?

<u>Afternoon session</u>

1. Px wears specs and is curious about CLs. What would you like to know?
    - What is it for? Football & rugby only. How long would they wear it for?
    - Spec rx: R+6/-2.00x140, L +6/-2.00x35 I FUMBLED BC THE POWER RANGE
    - What would you give? I can't mental maths for shit.
        - Said My day toric but then changed to acuvue then NO realised ONLY BIOFINITY WAS POSSIBLE SO I HAD TO CHANGE 2 TIMES. But would have to advise the px that it will be costly if they don't get the full wear out of them.
        - Biofinity Toric +6.25/-2.00x140, +6.25/-2.00x35 but then consider about the nasal torque as well. BUT MATH IT RIGHT - my calcs were not right. And lens parameters…
    - But patient said they will wear it only for sport? Too bad the only lens parameters available are monthly lenses. BUT i should've said that maybe the px can use it for full time, considerations is cleaning regimen and discomfort initially.
    - Is this patient suitable for RGP? Yes but will get by with SCL. Concerns with football and rugby being a contact sport so RGP can dislodge. So would not give RGP if they want to use it for sport.
        - What if they wanted to wear it for full time?
    - What if I wanted to swim? Suggested ideally not to wear CL but understand it is a high rx. So i would just give DD spherical equivalent to get them by. Wear goggles, wait 15mins before removal (otherwise you will take the epithelium off as well) then throw away.

2. ***"I am coming to you because I want to wear CL's, my glasses are slipping down"***
   (No further instructions)
   I acted as if she was the patient so asked questions to get more info:
    - What do you want them for? Sport, I play rugby and soccer.
    - What's the rx? +6/00/-2.00x140 and +6.00/-2.00x140
    - How often do you want to wear them? 3-4 days

   Gave a CL recommendation for the patient, Biofinity Toric, and justified why I chose
    - What if I want to swim in the contacts? Why would you say?
        - Strongly advise not to, increases risk of infection that can be sight-threatening
        - What if they insist "i can't see without"
        - Take precautions: "swimming googles over CL and discard after"

        - “LH: But this will increase the cost to the patient”
        - I agreed, but argued health of the eyes is more important.
        - LH: ok what if they swim very regularly like weekly, what would you recommend
        - No other options with SCL’s so need to stick with biofinity, can consider rx swimming goggles.
    - LH: What solutions can you use for monthly SCL’s
        - H202
        - MPS
            - Dug meself a little hole here: i said i would go for MPS cause he plays sport so may get more dirt deposition so rub and rinse will be better
            - LH: so you’re saying MPS is more effective?
            - I said well i think its better to remove any dirt or vegetative material
            - LH: so which is more effective clinically?
            - I said H202
            - LH: why?
            - I made something up about 3% H202 killing more pathogens than MPS lmao idk
    - Is this patient suitable for RGP?
        - Not really because of his lifestyle, high contact sport, increased risk of lens mislocation or blunt force injury
    - Can you name any RGP materials?
        - Boston XO, Boston XO2?, Menicon Z
    - What are important parameters? I said Dk, wettability angle and refractive index
    - Whats the difference between RGP cleaners and SCL cleaners
        - I talked about AOsept
        - Boston Advance Cleaner vs SCL MPS

3. Same case as above:
    - 20 yr old male wants to wear contacts and move on from spex. Plays sports.
    - Asked what sports: Rugby, Soccer.
    - Does he want to wear them full time or just for sports: Full time.
    - Whats his rx: 6/-2.00 x140 6/-2.00 x45
    - What CL you recommend:CV Biofinity Toric Monthly lens → why this lens and not others → only one that comes in >+6.50 and accurately corrects his cyl and since he plays sports want to maximise his vision instead of going with CV MyDay toric with not accurate rx. (OK Moving ON)
    - What if he want to swim? I said can do Daily disposable spherical equivalent MyDay Sphere for swimming. (OK moving ON)
    - RGP Suitability?
    - What RGP Materials? What factors you consider?
    - Are higher dk/t RGP thinner or thicker?
    - If you gave him the RGP what u expect to happen coz of his script? High plus thicker lens can ride down - How would you fix this? I said steepen peripheral curve.
    - What are some RGP Solutions? MPS vs Peroxide, Conditioner. Dug myself a hole with Menicon Progent and she asked what it does and started confusing myself here.
    - What if px want to sleep with RGP?
    - What are complications of sleeping with RGP?

4. Same case as above
    a. Questions to ask: worn CLs before? How often will you wear? What will you use them for? Dry eye/allergies/inj/surg?
    b. +6.00/-2.00x130 +6.00/-2.00x45 explained how to vertex correct
    c. Biofinity XR toric, said ranges for DS/DC/degrees. Said dailies toric don't provide that high for the rx
    d. Football/soccer, swimming? Ideally no swimming but if needed then wear goggles and make sure to wait before taking out CLs
    e. RGP suitability? Works with rx but may be difficult for physical sport which can cause lens to dislodge
    f. Complications of RGP? Mentioned hypoxia, 3&9 o'clock staining, impingement at limbus/vessels. She kept asking what else
    g. RGP materials?Name some. What factors would you consider? Dk, wettability. Dk for boston XO is 100, XO2 is like 140?
    h. RGP solutions? Name some. Peroxide vs MPS. Preservatives in MPS bad if allergic or hypersensitive. Peroxide good against fungi but needs 6 hours. Both bad against acanthamoeba.
    i. What would you tell the lab when ordering RGP? BC, Dia, BVP, Centre thickness. Said some other stuff like bitoric/BST needs 2 BC written down, and FST need + cyl.

5. Same case as above - 20 year old male student who wants to wear CLs for rugby/soccer and swimming in summer
    - Questions I asked: Previous CLs use? None. Any history of injury/infection to the eyes? None. How many days of wear? 3 days/week
    - Rx: R +6.00/-2.00 x 140, L +6.00/-2.00 x 35
        - Vertex corrected Rx to CLs, then told her that bc of the Rx DD SCL torics aren't available in that range → recommended monthly or fortnightly but did not discuss which ones nor a proper prescription LMAO (Would have told her monthly Biofinity Toric OR fortnightly cAir Toric and given her a proper CLs prescription as I only vertex corrected the SPH and CYL but didn't give her a proper axis)
        - Lily then asked me to choose between monthly and fortnightly so I chose fortnightly bc lenses would be more frequently replaced so this was better for the anterior eye health
    - Swimming: I said no CLs due to risk of acanthamoeba and pathogens but if they really must then DD CLs with tight fitted goggles, wait 15 mins after exiting the water and then remove. 2nd option I gave was prescription goggles.
    - RGP suitability
        - I said no bc although RGP give superior visual quality than SCL torics, RGP lens move on the eye more. Given that the patient is playing contact sports, RGP yeplenses can dislodge from the eye
    - Give the closest DD toric lens? THIS FUCKING THREW ME OFF so I bullshitted and said MyDay toric bc it goes up to +6.00 and covers -2.25 cyl
        - And in hindsight I realised it doesn't cover oblique axes BUT LILY ASKED CLOSEST DD TORIC :((((((((((
        - I did tell Lily that if the patient really wanted DD, then we'd have to compromise and set the expectation that the vision quality is less than expected

- Complications of wearing CLs with this Rx
    - I said that bc of the high plus, lens centre is thicker and can possibly decentre inferiorly on the cornea and create abrasions and discomfort + poor vision quality due to not looking through the optic zone
    - Also said that vision quality is not as good as spectacles, but said it was bc of the vertex distance (I think I'm wrong here)
- Cleaning solution systems
    - Talked about H2O2 and MPS and their indications - MPS is convenient bc it covers disinfection, cleaning, protein removal and involves rub and rinse and can be used for traveling BUT H2O2 is preservative free and better for compliance
    - How to choose between different MPS systems - I had no idea (went off track about disinfecting hours but realised manufacturers only list minimum soaking hours and then didn't have an answer for Lily)
        - Differences with rinsing lenses before wear → Biotrue and Optifree rinsing is optional, must rinse lenses with RevitaLens for 5 secs before wear
    - Problems with choosing the wrong MPS - said different MPS systems have different expiry dates so if you use it incorrectly or past the date then it's dangerous + not effective, not correctly performing the rub and rinse step as different solutions have different times?
    - In hindsight for the last 2 questions, Opti-Free has activity against acanthamoeba whereas the others don't so if you choose the wrong one, good luck

6. 20 year old male student who wants to wear CLs for rugby/soccer

- What do you want to know?
    - How many days per week? 3-4
    - Would you be open to wearing them full time? Maybe
    - Have you worn them before? No
    - Pertinent ocular + medical history? Medications? Trauma? Smoking? Nil
    - What are your expectations when it comes to CLs? (idk why but i asked how clearly do you expect to be able to see - she was like .. "as well as i can i guess" HAHAHA)
- What lenses are you thinking?
    - Given the risk of acanthamoeba i would be inclined to do a DD but if theyre receptive to wearing them full time, we could fit them with a monthly lens
    - (then i realised i hadn't asked for the script lol so i was like "BUT this is all contingent on their script, what is their script…?"
- Script is: +6.00/-2.00 x 140, L +6.00/-2.00 x 35
    - I said VD adjusted is R: +6.50/-2.25 X 130 L:+6.50/-2.25 X 40 and give Bioinfinity toric monthlies
        - I said i would preemptively anticipate nasal rotation in accordance with LARS, but i would want to fit them with the contact lens and assess rotation and make adjustments as needed
- Why are you worried abt acanthamoeba?
    - Theres a high risk of trauma and coming into contact with vegetative matter when playing rugby, either from being tackled or getting hit with the ball
        - Only now did i realise vegetative matter is a risk factor for fungal keratitis not acanthamoeba, and idek w vegetative matter means?? Is it just dirt?? Or dirt with vegetables??
- What contact lens solution would you recommend?

    - Optifree MPS because its shown to be effective against acanthamoeba
        - Here i go again w the acanthamoeba rip
- Why would you not give hydrogen peroxide?
    - I guess I could give HP, AOSEPT works with both soft and rigid contact lenses
    - But 2-step peroxide solutions have been shown to be effective against acanthamoeba, whereas most of the ones available on the market are 1 step which arent as effective
    - I said boston advance is rly good bc it has a surfactant, abrasive cleaner and alcohol based cleaner which is good for removing lipids, proteins and deposits
        - Thinking back now I think I insinuated I would use Boston Advance for a SCL rip but she didn't clarify w me
- Can you still fit them with a DD?
    - I said I can but this is why I clarified their expectations earlier, because we could get a DD but trade off on quality of the vision, for greater convenience of a DD
- What DD would you give?
    - I took a while to think and settled on MyDay toric. I said we'd have to give them +6.00/-1.75 x 20 (bc that was the closest)
        - Thinking back now i realise i could've given +6.00/-2.25 x 20
- Is this px suitable for RGPs?
    - For general wear yes, for rugby no
- Why for general wear?
    - He's got a significant amount of cyl that would be able to be helped with a spherical or BST toric → then i changed my mind and said the type of RGP would depend on how much corneal/internal astigmatism there is
- What problems can you see happening with this lens?
    - It would be low riding because of the high plus
- What would we do to fix it?
    - Its low riding bc the centre of gravity is pushed anteriorly bc its a high plus, to bring the centre of gravity back we can steepen the lens or increase the diameter
- Do you know any RGP materials?
    - Boston XO and XO2
- What are the differences between them?
    - Wettability and Dk
        - I said idk if we can say dk/t bc the thickness of the lens will change (ie with prism in FST etc) but we know the oxygen transmissibility of the material
- What complications can you see happening with RGP that wouldnt with SCL?
    - I guess 3 and 9 o'clock staining, and youd be more inclined to get mechanical issues. Like CLPC would be more of a localised CLPC rather than generalised
- Ok cool thats all we have time for

7. Same case as above: I'm a 20yo male who's wearing glasses but wants to try CLs for sport

- What do you want to know?
    - Have you worn CLs before? No
    - Would you want to wear the lenses for any other activities? No, just sport
    - What sports will you be playing? Football, soccer
    - Do you have any problems with dryness or allergies? No
- Rx: R: +6.00/-2.00x140 L: +6.00/-2.00x35
    - Vertex corrected to R: +6.50/-2.25x140 L: +6.50/-2.25x35
- What lens would you give?

    - DD would be ideal for occasional wear, but no plus powered daily lenses with oblique axis so would give cAIR toric
- What would you say if the patient wanted to swim in their lenses?
    - Advise not to swim in lenses, higher risk of infection. Should swim with Rx goggles instead
- What other options are there?
    - I asked how often the patient would be swimming - she said weekly-monthly - I said could swim on last wear of fortnightly CLs with tight-fitting goggles then throw away
- Would this patient be suitable for RGPs?
    - May have trouble getting used to them, but Rx is appropriate for RGP lenses. Ultimately no because of risk of losing lens during sport
- What are some RGP materials you know?
    - Boston XO, Boston XO2
- What's the difference?
    - Boston XO2 newer, better Dk
- Why do you think you see more Boston XO being used? (I mentioned I see Boston XO around a lot)
    - Maybe the consults were from before Boston XO2 was available (not sure how new it is), maybe practitioners prefer the more familiar material. Personally I would give people XO2
- What else is important with RGP materials?
    - Wetting angle - lower number is better
- What are some RGP cleaning options?
    - MPS, solution sets like Boston Advance, hydrogen peroxide cleaners
- How do you choose what solutions you want to recommend for soft lenses?
    - High water content hydrogel not suitable for H2O2, beyond that you decide based on risk of water exposure or how long the patient will be letting the lenses disinfect
- Let's say your patient is now 40 years old and having trouble reading, what lens would you give them?
    - Biofinity toric MF (got myself confused about what sph powers it was available in but said should be okay)
- What design is that lens?
    - Progressive lens with smooth transition of power between centre and periphery, comes in choice of centre near or centre distance
- How do you choose centre near or centre distance?
    - Forgot how the fitting works but eventually said just follow Coopervision's fitting guide and assess dominance

8. 20 yo, wants to try CLs for sports
    - Asked gender (F) and lily asked me why i wanted to know the gender :(((, i was like males of ~20yo are more exposed to the risk of getting corneal infection from cls due to inadherence idk if its true xd
    - What sports: rugby & football, 3-4x a day.
    - Planning to wear cls only for sports, but may wear more often in the future
    - Rx: 6.00/-2.00x140, 6.00/-2.00x35
    - No oc/medical history, no dry eye symptoms
- Which lens? Biofinity 6.50/-2.25x140 6.50/-2.25x40
    - Can give daily but decided to give monthly so that pt can wear them more often if he likes as per H&S

    - She asked me what is best for the pt; i said we can give daily for convenience & price efficiency but vision would be poorer. I think she might have wanted me to say DD w/ compromised rx, since u dont need 100% vision for sports idk
    - She asked me what i think about RGP for this pt → lens too thick & heavy so likely to decentre. Not ideal for sports. Also worried about hypoxia response from lens thickness
- Which solution for RGP? → H2O2 platinum disc based, Menicon Plus Multipurpose, Boston Advanced
    - How would u choose which one? → H2O2 straightforward & PF, Multipurpose straight forward & less hrs of contact compared to H2O2, less risk of burn, Boston Advanced: again no contact hr required other than rub and rinse
- Diff rigid materials? → Boston XO, Menicon Z, Paragon HDS
    - How would u choose which material to go for? → uhh i said dk, wettability, durability/hardness, refractive index, transmittance. I also said some of the designs come with a fixed material so u cant change it
- Pt turns 40 & requires reading add
    - I gave a few options - push for plus since u expect pt to manifest more hyperopia by this age, Biofinity toric MF w/ same spherocyl and +1.00 add centre near design, but it might be a bit of an overkill when pt is only 40yo, monovision w/ +0.50 more on non-dom eye

9. Literally shat myself ahjhj - 20 yo sport, rugby soccer player.

- What would you ask them:
    - Does shift work wants to wear CLs 3-x/week for sports mainly.
    - (-) dry itchy sore eyes, meds, allergies, infections, surgeries, trauma, diabetes
- Spec Rx: R +6.00 / -2.00 x130 L +6.00 / -2.00 x 35
    - CL VD Rx: R +6.50/-2.25 x 130 L + 6.50/-2.25x35
- What lenses would you recommend and why?
    - Biofinity toric +6.50 / -2.25 x 130 +6.50 / -2.25 x 30
    - Because you get the power and axis and cyl in this unlike most of the DDs
- What about DD?
    - Myday Toric: +6.00 / -2.25 x 120 +6.00 / -2.25 x 30
      Screwed up the axis tho
- When would you recommend RGPs?
    - Wearing lens all day not during sports
- Swimming?
    - Would advise against due to infections
    - But if persist then would recommend DD.
- Complications with RGPs with this particular lens?
    - Low riding due to hyperopic prescription.
- What RGP materials are there and what would you recommend to your px?
    - Fluorosilicone acrylate and silicone acrylate
    - Would recommend Boston XO as fluoro has higher Dk/t

FRIYAY ALMOST THERE KIDS ….

TOPIC PREDICTIONS?????

- RGP troubleshooting
- Keratoconus? & corneal ectasias - i will commit - nooo ur preciousssss <3
- Noooooooooooo - illllllll dieeeeeeeeeee
- *Good luck everyone :c*
- *Ay lmao*
- *Is there anyone going at 9am? - asking for a friend :)))) <3<3<3*
- *Super stressedddddd early colleagues pls put something here - im at 10 i will put it here straight away - legend thx so much! Good luck <3*
- *ThankHEL yo usssssssso much babe love you*

We all just waiting huh yup
Plz guys xx AHAHAH plssss & good luck to yall :') yup good luck everyone!
I am shitting brix dude i be shitting houses
What happened to the 10am student :'(:'(
Nearly crying

OMG YOU'RE HERE I LOVE YOU I confess my everlasting loveee

<u>Morning session</u>

- Okay so I got a script **+5.00/-0.75x120 RE & +5.00/-1.25x10 LE**
    - **When i do the calcs I get +5.25/-0.75x120 and +5.25/-1.50x10? Yes that is what i gave :) yaay! Consider that MyDay doesnt come in 120 though so maybe mention that would give 110 axis**

- 40 year old patient who is looking for contact lenses to use whilst scuba diving
- What contact lens options do I have? SCL vs RGP - fortnightly and monthly not suitable because you would want them to dispose of lens immediately after
-
- What script would you give?
- What lens would you recommend for this patient? I initially incorrectly said Dailies Total 1 for astigmatism but corrected myself because it only goes up to +4. Changed my answer to Coopervision MyDay Toric lens. (i mentioned axis not being available for RE but she never probed further on this but you can compensate for this using LARS)
- Then I got grilled on Dailies Total 1 and why it was my first choice
    - Why is it better? Highest Dk/t, water gradient technology, comfortable. I mistakenly said that it has been shown to be successful and comfortable. SHe kept asking who said this, how did they measure it….. Mate idk hahahha alcon said it
    - Does that mean Coopervision MyDay is not good? Sorry what ahhahaha
- Then she said what would you be worried about? I said hypoxia would be my main concern
- Why is the patient more at risk of hypoxia? Very minimal to no O2 exposure whilst scuba diving - contact lens barrier to O2 + mask and goggle barrier. High pressure environment also.

2. Same as above lol.
Went for DT1 astig but changed it cuz of power range. Asked alot about modulus and how it can affect the lens you choose. I screwed up cuz i changed it to myday and then realised it afterwards like it wasnt in the power range too AHAHHA but i changed it again not sure if lily liked that. (my day is in range isnt it?) i think it is +4 for the myday torics? So it wouldnt reach the +5.25 rx? Im not 100% sure. Oh wait nvm then. Oops. oh wait i think i mixed it up oh well. Then talked about oxygen and hypoxia.
MyDay is +6.00 I believe i think so too? Only DD toric up to +6. Yea i think it is. I got it mixed up rip. Aww dw. I did mention it tho before stabbing myself by correcting it so hopefully its like brownie points AHHAA.

- **Do you guys talk about nasal tortion so ????**

3.okay Same as above
- I got a script **+5.00/-0.75x120 RE & +5.00/-1.25x10 LE**
- Also gave 110 axis. In hindsight, maybe mention to give -1.25DC since you can't do 1.50DC as well

Same as above– after she asked about if the px wants to wear CL and is struggling wear near vision:
- Also asked what you want to tell the px after they finish snorkelling (didnt get scuba diving):
    - I said wait 15 mins to not rip off the corneal epithelium, and can use lubricants after if struggling
    - Signs of red eye and infection
- **She also asked CV MyDay range so be careful out there**
- I was like - what's his near add and she was like what do you think lol omg omg -> **could we just go looooow add?**
    - **I did low add and said DT1 MF says up to +1.25 add so would be ok, and said Bionfinity Toric MF is okay but starts at +1.00add**
- Anyways I discussed the options of overspecs on his CL, or spherical equivalent on ADT1 MF, or CV Biofionity XR
- Then she said what other options and I said monovision
- Asked how i would assess then explained dominance tests then she said cool cya

Good job!!

4. Same as above
- **Considerations for scuba diving**
- **What are you concerned about**
    - I said RGP lenses are more susceptible to suction when atmospheric pressure increases (I googled that) but dug myself a hole cos she kept asking why
- **What lens would you choose** MyDay Toric
- **What script? How did you choose the cyl script?**
- **What would you tell th ve px about scuba diving and SCL**
    - DD better, wash eye with saline before removing lens or wait 10-15mins first, risk of infection, return for review ASAP if infection or irritation
- **What are you looking for when assessing SCL fit?**
    - Centration, movement, tightness, comfort, vision
    - **What is an optimal movement?**
        - 0.20 to 0.50mm??

- **Px reports difficulty with reading, what would you give him now?**
    - I said Monovision first → **why?** → better for steropsis, lower ADDs easier to adapt, easier to troubleshoot as single vision lenses
- **What other options?**
    - Can do MF CL. I said DT1 MF, can give LO add.
- **What script?**
    - I'd just give +5.25DS OU
- **"Are you just going to abandon the cyl???"**
    - I said yes and then was going to say something about 4:1 rule, sph is enough to mask the cyl here. But we ran out of time and she said bye :(
    - Is 4:1 only works as minus ? -> so same direction of cyl and spherical
        - FK really, i did not know that
        - Oh you are right. -4.00 masks -1.00. Welp i forgot my SCL MF Torics anyway.
        - Wait so i guess we just say spherical equivalent?
        - IF GO DD, MAYBE equivalent - if go biofinity, go toric ? - this is only available in monthy
        - Nah I would say CVH Biofinity MF Toric (monthly) for scuba? **She said "what if the px wants to wear the lenses everyday" I think diff scenari oooo i see**
- **GOOD LUCK!!!**

MY DAY TORICS COME IN THAT SCRIPT??? yes
https://coopervision.net.au/practitioner/our-products/myday-family/myday-toric - THERE U GO KIDS

Replacement schedule	Daily		
Oxygen transmissibility	80 Dk/t (at -3.00D)		
Thickness	0.1		
Revenue carton size	30-day, 5-trial		
Design	Optimised Toric Lens Geometry™		
Base curve	8.6	8.6	8.6
Diameter	14.5	14.5	14.5
Sphere power	Plano to -6.00DS (0.25D)	-6.50 to -10.00DS (0.50D) +0.25 to +6.00DS (0.25D)	Plano to -6.00DS (0.25D) -6.50 to -10.00DS (0.50D) +0.50 to +6.00DS (0.50D)
Cylinder power	-0.75DC -1.25DC -1.75DC	-0.75DC -1.25DC -1.75DC	-2.25DC
Axis	10 to 180 (10° steps)	10°, 20°, 70°, 80°, 90°, 100°, 110°, 160°, 170°, 180°	10°, 20°, 90°, 160°, 170°, 180°

5. Same as above

- **40 year old scuba diving RE +6.00/-1.00x10, LE +6.00/-1.25 x10**
- **Asked what i would prescribe?** Ahhhhhh i shouldn't have said My Day (dug a hole):). But showed her my spherocyl power cross on paper
- **Considerations for scuba diving, what i would tell my patient?** Same spiel about waiting 15 mins before removing CLs and using lubricants to loosen adherence. Prevent epi ripping off. Also toric lens so worried about hypoxia.
- **Why else would i concerned about hypoxia? Lot of what else?**
- **Also about the difficulty seeing up close, what i would prescribe?** Considering monovision first since early presbyope, so would require low Add. Easy to adapt to once trained in clinic and assessed suitability.
- **Is monovision or MF CLs better for binocularity?** Not sure, i did say monovision at first then changed to MF. Asked why
- **What MF SCL Toric is available in the market?** Mentioned Bioinfinity and Proclear torics.
- **Which MPS solution would i recommend if the patient decides to wear CL everyday (so in a reusable CL)?** I said optifree pure moist since has citrate as active ingredient and compatible with most CLs in market

6.!!!! Omg GUYS SHE CHANGED THE PRESCRIPTION !!!

- **40 year old scuba diving RE +6.00/-1.00x10, LE +6.00/-1.25 x10**
    - So guys please go directly into **biofinity**
    - i got final rx 6.50/-1.25x120, 6.50/-1.25x10 - my badddd math - then Ask me how to do the calculation for cyl lol
    - **Considering diving, what you need to tell them** -> mask + glass + take off after 15 mins + if struggle can use saline to thorough rinse.
    - **Then she ask me what you need to think about the lens for diving** - i say better disinfection , then she ask me which solution i go **AOSEPT**.
    - **Then she ask me why do you go this** -> i say easy to get and hydrogen peroxide has been approved better disinfection compared to chemical one. Then she ask me **what the main bacteria risky for diving**. I was like WTF but say acanth + pseudomono -> then she ask me do you think the solution is sufficient for disinfection ? -> I have fucking no idea but remember there is one slide talking about acantho -> so H_2O_2 less than 1 hour denifityly no, for overnight disinfection is contravericial, most paper say no but some paper yes -> she didn't say but she is like "OK i know you have no idea about that let's move on. "
    - **I try to get myself back aww :((**- say DD may be a good choice for diving only but not available in the market, definitely better discard lens after diving but considering the cost, I need to explain to the px the benefits between disinfection vs discard. And it's critical to emphasis the importance of AC appointment
    - **Also she ask what's your main concern -** i talk about **hypoxia**, due to toric designs higher risk of hypoxia -> effect on cornea and how edema come from then how px feel (halo around the light) - i didn't do well bc i freaked out by the bacteria questions, guys pls prepare for hypoxia
    - **Then she ask** - px come back for AC and want to wear the lens more frequently, but have trouble with near - what you would go
        - I talk about spec over, then mono vision, then multifocal
        - She ask about **monovision and lens choice** -> i say check which eye is dominant first then correct with BCVA distant RX, non dominant have mild add

(about + 1~1.25D on top of current Rx) - fuck i just notice she ask about choice but i didn't gave any
- I was really looking for the question about how to check dominant eye but she didn't ask 😕
- I feel so bad as i keep repeating myself. Then She say that's all for today and good luck for the rest of the term - I don't know why but i freaked out :(:((im sure you did good! Nah you did amazing i woulda buckled so bad :))

Was there any history taking, or just right into qs?

I asked, I say if you wanna me to take more history questions or i just directly go to Rx -> then she ask me to gave her my Rx for CL and keep asking question then Thanks!

7: 40 year old scuba diving RE +6.00/-1.00x120, LE +6.00/-1.25 x10 (same as above)
First time CL wearer, no previous ocular infection
Convert to +6.50/-1.25x120, +6.50/-1.50x10, so suggest **biofinity toric** (she asked why, I said I want to give DD but not in this Rx, only can go to biofinity toric)
- I asked how frequent you want to wear it?
    - The whole week for scuba diving. Then I asked what about other time, do you want to wear it full time? She said I'll wear it if its good.
- **I asked her can you get an eye mask with Rx?** LOL
    Lily said: No, I want to wear CL → did she ask what you would do after every scuba diving session? Chuck vs clean? Chucking a toric every session is $$ I'm glad she didn't ask, ikr, personally i don't want to use my monthly lens once and chuck it… but we need to do that as an eye care professional i think. **Thanks hon.** Nws, good luck!!! She is nice!!!
- **Which solution would you suggest?**
    MPS (OptiFree 6h at least overnight)
- **Came back for you after scuba diving, want to wear CL full time, but have trouble reading**
    2 options: over-spec or CL full time(a.Monovision, sv, cheaper; b.Biofinity MF toric)

Then she let me go, good luck guys, we almost there!!!

8: same as above
- First time cl wearer, wants to go overseas trip scubadiving
- No remarkable hx
- R +6.00/-1.00x120 L+6.00/-1.25x10
- I gave her my converted rx, she asked which lens i would consider rx'ing i said biofinity toric if the px wants to optimise the rx (altho i gave her the converted rx and not the cl rx [considering the cyl is -1.50 in the LE] she didn't rlly ask??) -- **i prob should've made it clear the LE cyl needs to be adjusted**
    - **Omg also i dont remember if i specified wearing goggles OVER the cls fml i assumed it was a given but if she asks the same q remember to specify**
- Asked: is it impossible for this px to wear DD? I said no but it will compromise vision so if they're ok with that then that's fine since DD is best in the context of scuba diving
    - **Didn't** rlly ask about what to do after removal but maybe bc i was short on time
- Also asked: would you rx RGP? I said ideally not bc lens drop out due to smaller diam, thicker lens hypoxia
- What MPS would you use? I said optifree puremoist bc acanth (context of scuba diving)

- And then the px returns to you, wants to wear CL, trouble reading
    - Gave her the options of over spec, monovision, mfscl
    - She asked which mfscl? I said biofinity bc of the rx
    - Asked me how the biofinity design works → i said concentric rings, comes in centre dist and near
    - Asks me which i would rx the px and i fumbled i said it depends, considering pre-presbyopic if they use prioritise their dist vision more i would go centre dist, vs if they're in the situation of lots of reading with bright lights i would go near centre 'it depends' LMAO
        - **I think the answer is cD**

Afternoon session

- *Please o lovely humans LOL - I can write a prose: Roses are red, Violets are blue, but there ain't no one as amazing as you <333 aww cute* 🥲

1. First time CL wearer, wants CLs to go on holidays, everyday wear on holidays
    a. Hx of CL wear? Duration? Pre-existing corneal conditions?
        i. Spec Rx: -8.00/-1.00x120 -8.00/-1.50x30
            1. Corrected CL Rx: -7.25/-1.00x120 -7.25/-1.50x30
    b. What lens would you recommend?
        i. MyDay Toric or Biofinity Toric
            1. Which modality? Daily for convenience Why? don't have to worry about cleaning lens whilst on holiday, reduce risk of complications/infection
            2. What other daily lens could you rec?
                a. DT1 for astig; tell me the power availabilities and parameters
                b. Why did you pick MyDay then? "Sorry my mistake, i'd rather pick DT1 because it has better breathability esp if they have longer wear hours, i'll change my answer pls lils"
        ii. What would you advise px whilst they're on holidays?
            1. General handling hygiene
            2. Don't shower, sleep or swim in lenses
            3. Don't wear lens for more than a day (eg. don't save a lens to wear the following day)
            4. If you get any FB in the lens, take them out and dispose
                a. Pack more lenses than how many ever days you're travelling for so that if you get dirt/debris on lens you can dispose of it and still have enough to wear
    c. Px has come back after their holiday and they want to convert to fulltime wear. Would you change the lens?
        i. Monthly modality because cheaper if >5 days of wear per week
    d. Are RGP's suitable for this px? Why do you recommend soft CLs prior to RGP?

2. BASICALLY SAME AS ABOVE (25 yo first time CL wear, wants CLs to go on holidays, FULL TIME WEAR, likely to be swimming in them)
    i. I didnt ask about anterior eye cuz i forgot but she didnt ask so 🙂
    ii. SPEC RX (R -8.00/-1.00x120 L -8.00/-1.50x10)

a. First Q: What lens would you recommend
    i. J+J acuvue oasys for astigmatism (cuz first lens that i knew had those powers)
        1. Oh ye i said its SiHy which is cool for full time wear and high modulus so easy to handle cuz CL noob
    ii. OcularRx (R -7.25/-1.00x120, L -7.25/-1.50x10)
        1. R -7.00/-1.00x110 (bit different cuz availability of lenses)
        2. L -7.00/-1.25x10

        Arent they both full circle for myopes up to 1.75 dc
        NAHH not when u have -7.25sph then it gets all wack with the axis trust
        im dumb but ik this
        Oh boy

DAILY DISPOSABLE TORICS

- **CooperVision Myday Toric 8.6/14.5**
    - Plano to -6.00
        - Cyl -0.75, -1.25, -1.75 (Axis 10 to 180 in 10-degree steps)
        - Cyl -2.25 (Axis 10, 20, 90, 160, 170, 180)
    - -6.50 to -10.00 (0.50 steps)
        - Cyl -0.75, -1.25, -1.75 (Axis 10, 20, 70, 80, 90, 100, 110, 160, 170, 180)
        - Cyl -2.25 (10, 20, 70, 80, 90, 100, 110, 160, 170, 180)
    - +0.25 to +6.00
        - Cyl -0.75, -1.25, -1.75 (Axis 10, 20, 70, 80, 90, 100, 110, 160, 170, 180)
    - +0.50 to +6.00 (0.50 steps)
        - Cyl -2.25 (Axis 10, 20, 90, 160, 170, 180)
    - Modulus 0.4 MPa (low)
    - SiHy (Dk/t 80, 56% H2O)
- **Alcon Dailies Total 1 for Astigmatism 8.6/14.5**
    - Plano to -6.00
        - Cyl -0.75, -1.25, -1.75 (Axis 10 to 180 in 10-degree steps)
        - Cyl -2.25 (10, 20, 70, 80, 90, 100, 110, 160, 170, 180)
    - -6.50 to -8.00 (0.50 steps)
        - Cyl -0.75, -1.25, -1.75 (Axis 10, 20, 70, 80, 90, 100, 110, 160, 170, 180)
        - Cyl -2.25 (10, 20, 160, 170, 180)
    - +0.25 to +4.00
        - Cyl -0.75, -1.25, -1.75 (Axis 10, 20, 70, 80, 90, 100, 110, 160, 170, 180)
        - Cyl -2.25 (10, 20, 160, 170, 180)
    - Modulus 0.7 MPa (core, 0.001 surface) (high mod)
    - SiHy water gradient (Dk/t 140 33 to 100% H2O)
- **J+J Acuvue Oasys 1 DAY for Astigmatism 8.5/14.3**
    - Plano to -6.00
        - Cyl -0.75, -1.25, -1.75 (Axis 10 to 180 in 10-degree steps)
        - Cyl -2.25 (10, 20, 70, 80, 90, 100, 110, 160, 170, 180)
    - -6.50 to -9.00 (0.50 steps)
        - Cyl -0.75, -1.25, -1.75 (Axis 10, 20, 70, 80, 90, 100, 110, 160, 170, 180)
    - +0.25 to +4.00
        - Cyl -0.75, -1.25, -1.75 (Axis 10, 20, 70, 80, 90, 100, 110, 160, 170, 180)
    - Modulus (0.72) (high)
    - SiHy (Dk/t 129, 38% H2O)

Kinda messed ngll
Actual jks but DT1 is alternative has the power ranges but still no 120 cyl
Jks myday also works
Im dumb as FCK (i said myday only went up to -6 (but it goes up to -10))

b. What advice would you give
    i. General hygiene (wash hands, clear environment thats not bathroom)
    ii. Dont shower, sleep or swim. If you must go swimming wear goggles
    iii. Dispose daily
    iv. If any red,itchy,irritated eyes remove dont wear lens and go to optom. Bring your glasses as well in case.
    v. She wanted more but my brain was dead, probably more stuff related to the traveling idk (dry eye on airplanne???, bring more lens wtffff)
c. She loves the lenses!! So she wants to wear Ft after holidays but cheaper 🙂
    i. Gotta go monthlies yo
    ii. I said Biofinity toric cuz only monthly ik but i justified it saying it came in the 120 axis which wasnt avail with the acuvue oasySUs
        1. R -7.00/-0.75x120
        2. L -7.00/-1.25x10
    iii. Didnt really question anything else cuz ran outta time
        1. But basically monthly advantage (it cheap, it come 120 axis)
            a. But u gotta clean :(

3. Same as above SPEC RX (R -8.00/-1.00x120 L -8.00/-1.50x10)
    a. Convert to ocular rx R -7.25/-1.00x120, L -7.25/-1.50x10
    b. What lens would you choose?
        i. I gave 2 options Myday toric or DT1 DD (in hindsight just specify 1 otherwise shes gonna probe why you chose one over the other)
        ii. I chose DT1. why
    c. What advice would you give?
        i. What diff advice would you give for a general px vs one who is going on holidays?
    d. Patient comes back from holiday. Wants to wear full time.
        i. Change to monthlies bc cheaper if wearing everyday
    e. What cleaning solutions would you give? Why this solution specifically
        i. MPS - optifree purevision, Biotrue, revitalens
    f. Would you give RGP to this patient? Why didn't you consider it before?
    g. Said there are diff toric RGP options but need to consider corneal astigmatism as well to decide which to go for
    h. What do you expect corneal astigmatism to be? Idk what she meant

4. Mine was very different, mostly about RGPs
    a. First thing she asked me was about advantages and disadvantages of RGPs

    i. I said RGPs last longer, easier to clean so less complications such as CLPC, can give superior vision in cases of cornea ectasia conditions
    ii. Disadvantages: easily dislocated when doing physical activities, less comfortable,
b. Can you name some RGP materials, what do you look for when deciding on an RGP material. I said fluorosilicone acylate and about dk/t but i think she wanted me to say more
c. Name some RGP trade names. I said Boston XO2
d. Then she asked me about soft CLs materials and what do look out for
    i. Talked about SiH and H material and how SiH is stiffer compared to H. Also talked about modulus, water content, thickness etc.
e. Then gave me this spectacle rx: R: +2.00/-6.00x20 L: +1.00/-4.00x10
    i. Convert it to CLs rx: R: +2.00/-5.75x20, L: +1.00/-4.00x10 (hopefully that's right?)its right! :)
f. What options CLs options do you have for this rx
    i. For soft CLs I said biofinity toric XR
    ii. For RGPs I said it will depend on the K values. Then she said just in general and I said bitoric because with this amount of cyl there is probably a lot of corneal cyl
    iii. Then she asked which one is better I said RGPs were better because soft toric CLs aren't very good for high cyl and low sph scripts. Also the RGP will probably give better vision because it will correct the corneal cyl
g. Then we ran out of time and that's it

She completely changed it. RGP lenses.

1. Advantages of SCL compared to RGP (ngl i cant even rmb if she asked this or not, maybe i imagined it).
   SCL more comfortable, can be disposable, less initial cost, don't have to clean it if dailies, rigid lenses have more mechanical effect and can cause CLPC → less risk in softer lens.
2. Advantages of RGP compared to soft.
   Better vision bc it masks the irregular cornea, good for irregular corneas, lenses more durable, lenses can last up to 2 years, less risk of infection.
3. When would you use RGP.
   High corneal astigmatism, irregular astigmatism.
4. What effects can you get from RGP.
   Hypoxia, if lens is decentered can cause corneal indentation, uncomfortable bc of lid interactions, ngl i cant rmb what i said, i think i just explained each point and how it happens.
5. She asked about what RGP lens materials there are and what are the differences between them, what would make u pick one over the other.
   RGp comes in fluorsilicone and siloxane. Different RGP materials are Boston xo, boston xo2, fluoroperm, menicon z, then come if difference dk/t. All of them come in spherical,

aspherical, FST, BST, bitoric, bifocal, and MFs, except menicon Z it doesnt come in bifocal.

6. Gave me an rx +2.00/-6.00x20 and +1.00/-4.00 find the BVP correction.
7. What lens would you give to this pt → i said depends on corneal topography whether its more internal or corneal astigmatism (i was assuming it was still RGP, but then she asked so are you gonna od RGP on this pt, and i said yh itll give them better vision bc the back surface will be more customised to the cornea if there is high corneal astigmatism)
8. She asked something about do RGP always correct the high corneal astigmatism:
   I said no, bc if you're fitting them w a bst you also have a toric tear lens and then leads to induced astigmatism, so it doesnt fully correct it, you would have to do a bitoric lens.
9. Then she asked why wouldnt you use RGP:
   If the pt has little corneal astigmatism, if the pt wants comfortable lenses, if the pt doesnt want to clean them, if the pt is old and their lids aren't gonna hold up the lens (ectropion, entropion, ptosis).
10. Then she asked if you were to give SCL what would it be i said i buckled between proclear xr and biofinity xr, but then went with proclear, cause i think +2.00/-5.75x20 is only available in proclear xr?
11. Then she said why no biofinity xr, and then i said bc it doesnt come in +2.00 with a -5.75DC. I thought it does come in +2.00 in biofinity toric XR?
    I think for biofinity xr it only comes in +2.00 for 0.75 to 2.25 DC. if you want -5.75 cyl its only +8.50 to +20.00.
    https://coopervision.com/practitioner/our-products/biofinity-family/biofinity-toric-biofinity-xr-toric
    Idk it says +/-20D omfg ur right, soz i fr got confused during it All good!
12. Then she asked what CLs solution would you give, i said opti-free puremoist bc good anti microbial effects and readily available.
13. Then she asked why not hydrogen peroxide, i said u could give hydrogen peroxide, but i would lean more towards hydrogen peroxide if the pt had more water contact bc they are better for acanthamoeba cysts whereas puremoist is only for trophozoites, not good for cysts. Ultimately depends on the situation, both are good, but i would do h2o2 if the pt actually needed stronger disinfecting solution.

She asked whack qs y'all

1. Advantages and disadvantages of RGP lenses vs SCLs
   a. RGP lens: Adv: clear vision, better tear exchange, less deposits and debris
   b. Disadv: RGP less comfy, easier to dislodge, lens awareness and increased adaptation period
      SCL: more deposits/debris
2. Name some RGP lens materials
   a. Fluorosilicone acrylate (Boston XO dk 100, XO2 dk 140) and silicone acrylate (Boston 2, 4 with dk 12-19). Fluoro increases the oxygen permeability.
   b. Which material would you choose? FSA
3. Difference between SiH and H?

    a. SiH: higher dk/t, lower water content and higher modulus (stiffer) than H lenses.
4. What are some examples of hypoxia on the cornea and other layers?
    a. Cornea: vacuoles, microcysts, striae, folds
    b. Endothelial - blebs, polymorphism, pleomorphism
    c. Worse effects neovascularisation
    d. MY CONNECTION BROKE HALFWAY THROUGH TT-TT
5. How would you check for endothelial hypoxia?
    a. Specular reflection with 40x mag
6. Spectacle Rx: R +2.00 / -6.00 x 120 L: +1.00 / - 4.00 x 30
    a. VD conversion: +2.00/-5.75x120, L: +1.00/-4.00x30
7. What lens would you give the px?
    a. Biofinity toric XR monthly +2.00/-5.75x120, L: +1.00/-3.75x30
8. She would RGP be better than SCL for this px and why? What lens would you give
    a. RGP tear lens corrects the astig, soft CL with high cyl is not that great, if any minor rotation could affect the vision significantly
    b. Bitoric lens as likely high corneal astigmatism
9. And then she started asking qs about internal astigmatism and if soft CL corrects it and i knew shit. Does anyone know??
10. And she asked me how a RGP is stablised and whether an RGP or soft CL would rotate more?? idkkkk